

IN THE MATTER OF AN ARBITRATION
BETWEEN:

THE TORONTO TRANSIT COMMISSION

(The “Commission” or the “Employer”)

AND

THE AMALGAMATED TRANSIT UNION, LOCAL 113

(The “Union”)

AND IN THE MATTER OF THE GRIEVANCE OF ANTONIO GEORGOUDAKIS
CONCERNING THE DENIAL OF LTD BENEFITS

JANICE JOHNSTON - SOLE ARBITRATOR

APPEARANCES:

For the Employer:	Dolores Barbini	Counsel
	Patricia Matusiak	Counsel
	Amanda Lawrence-Patel	Counsel
	Dave Skwira	E.R. Consultant
For the Union:	Alanna Mihalj	Counsel
	Bill Chrisp	Executive V.P.
	Antonio Georgoudakis	Grievor

Hearings in this matter were held in Toronto on: July 11, September 19, November 2, December 12, 2016; February 13, March 6, April 10, 13, 21, May 17, June 9, September 20, October 17, 18, 24, November 13, 27, December 12, 21, 2017; April 23, 30, October 12, 19, 23, November 5, 2018.

AWARD

BACKGROUND

The grievance in this case asserts that the employer, the TTC, failed to provide long-term disability benefits (LTD) to the Grievor, Mr. Antonio Georgoudakis, during the own occupation period, contrary to the Collective Agreement and the Human Rights Code. The Union is seeking:

1. A declaration that the TTC has breached the Collective Agreement and applicable legislation.
2. A declaration that Mr. Georgoudakis is entitled to LTD benefits in accordance with the Collective Agreement.
3. An order that the TTC pay to Mr. Georgoudakis the LTD benefits, plus interest, to which he was and is entitled in accordance with the Collective Agreement.
4. An order for damages for lost peace of mind.
5. An order for damages arising from the violation of Mr. Georgoudakis's human rights, including damages for injury to dignity, feelings and self-respect.
6. Any other relief the Union should request and an arbitrator may grant.

The grievance is dated June 11, 2015, and was referred to arbitration on January 21, 2016. The period of time for which benefits are sought is December, 2011, to December, 2013.

The Collective Agreement provides in Article 1, Section 18 that the TTC agrees to provide all employees on Long Term Disability benefits in accordance with the Letter of Understanding (the LTD "LOU") dated September 23, 1987. That document is long and I will set out the relevant portions of it later in this decision.

The circumstances that have led up to the writing of this decision are somewhat unusual. As can be seen by the number of days spent litigating this case, it was a very lengthy proceeding. It commenced on July 11, 2016, and ended on November 5, 2018.

The Grievor's employment was terminated on November 1, 2013, and the parties invited me to act as sole arbitrator to determine that grievance by letter dated February 19, 2014. The hearing in the termination case commenced on June 25, 2014, and concluded on May 14, 2018. On August 9, 2018, I issued an eighty-page decision upholding the termination.

Mr. Georgoudakis was terminated for failing to medically substantiate his ongoing and long-term absence from work with objective medical documentation; abandoning his employment; and failing to maintain regular contact with Divisional Management. Therefore, although the issues raised in the termination case are not the *same* as in this case, in which an improper denial of LTD benefits is alleged, there is some overlap between the two cases, both in terms of the issues and the evidence. The bulk of the medical evidence before me is the same in both cases. Therefore, in particular, some of the factual conclusions I reached with regard to the medical evidence in the termination decision are relevant to this case.

The situation is further complicated by the fact that the Grievor chose *not* to testify in the termination case and the Union did not call any evidence. Accordingly, all of my factual conclusions in that decision are made on the basis of documentary evidence or witnesses called by management. In the LTD proceeding, Mr. Georgoudakis provided lengthy testimony and was in the witness stand for fourteen days. Accordingly, it may be necessary for me to review and potentially change some of the conclusions I reached in the termination decision based on the testimony of Mr. Georgoudakis. It is possible that I might come to a different conclusion based on the different evidence in this case.

However, having said all that, many of the overlapping facts between this case and the termination case are not in dispute.

The Grievor commenced his employment with the Commission in July, 1994, as a bus operator. He became a subway operator in 1996. After becoming a subway operator, the Grievor sustained workplace injuries, some of which were physical in nature and others that had psychological consequences. As a result, he has a long history of absences from work during which he collected Workplace Safety and Insurance Board (WSIB) benefits on numerous occasions.

The first incident causing Mr. Georgoudakis traumatic mental stress occurred on June 26, 1997. A young man committed suicide by jumping in front of the train he was driving at Coxwell Station. Mr. Georgoudakis saw his family doctor in Mississauga, Dr. Peter Findlay, and received counselling on June 26, 1997, and July 8, 1997. On December 12, 1997, he was working as a guard on the train when a man jumped in front of the subway train at Castle Frank Station. On August 12, 2001, Mr. Georgoudakis was working as a guard when a man jumped in front of the subway train at Jane Station. On August 13, 2001, he saw Dr. Findlay, who noted that the Grievor was stressed out and feeling anxiety.

On March 29, 2002, Mr. Georgoudakis was working as a guard when a man spat in his eyes and shouted, "Welcome to the world of AIDS and hepatitis." He saw Dr. Findlay, who sent him for testing for infectious diseases and provided counselling during appointments in April, May and June, 2002. The results of those tests came back negative. Dr. Findlay also noted that Mr. Georgoudakis was suffering from symptoms of post-traumatic stress disorder (PTSD) and anxiety.

On October 21, 2003, Mr. Georgoudakis was working as a guard when he witnessed a brawl at Main Street Station, including seeing a man beating a woman with a stick. Two of the perpetrators came up to him when he was reporting the incident to transit control and told him that they would kill him if he reported the incident. Later that day, a man threw a brick or concrete block at him at Ossington Station. It just missed the window,

and Mr. Georgoudakis injured his knee when he ducked and fell. The Workers Report of Injury (WSIB Form 6) indicates that he saw another family physician, Dr. George Zachariadis, on October 22, 2003. Mr. Georgoudakis saw Dr. Christos Soulios, a psychiatrist, on December 3, 2003. Dr. Soulios noted that post-traumatic stress disorder was his clinical presentation and that major depression needed to be ruled out.

On March 9, 2005, Mr. Georgoudakis fell down the stairs at Royal York Station and hit the back of his head on the stairs. He saw Dr. Findlay after this injury several times between March and November, 2005. Dr. Findlay noted he had post-concussional headaches and subsequently diagnosed him with anxiety.

On October 1, 2006, Mr. Georgoudakis was working as a guard when a man jumped onto the tracks at Chester Station. The driver managed to stop the train before contact, but the man remained at track level by the electrified third rail. Mr. Georgoudakis ran to cut the power to the third rail, and did so just before the jumper touched the rail. Mr. Georgoudakis saw Dr. Findlay on October 4, 2006. Dr. Findlay noted that Mr. Georgoudakis was “re-stressed” and reported flashbacks. He booked him off work.

On February 21, 2007, Mr. Georgoudakis was involved in a workplace incident that resulted in his being off work from that date until July, 2010. He was performing guard duties when a passenger threw a water bottle at him. He suffered physical injuries and psychological trauma. His leave was approved by the WSIB.

On February 21, 2007, he saw Dr. Zachariadis, who recorded that Mr. Georgoudakis was suffering from an anxiety state and phobic reactions. On March 12, 2007, Dr. Zachariadis reported that Mr. Georgoudakis was suffering from an anxiety state, panic attacks, phobic reaction, insomnia and nightmares.

Mr. Georgoudakis saw Dr. Soulios again on May 9, 2007. Dr. Soulios wrote that PTSD needed to be considered. Mr. Georgoudakis also attended an independent medical examination (IME) on February 5, 2008, with Dr. Donald Young, a psychologist. Dr. Young made a primary diagnosis of PTSD, along with a provisional diagnosis of major depressive disorder. Mr. Georgoudakis received treatment from Dr. Joanna Mitsopoulos, a psychologist, between June, 2008, and June, 2009. She diagnosed him as suffering from PTSD and major depressive disorder (moderate).

As part of his treatment program, he was referred to the Psychological Trauma Program (PTP) at the Centre for Addiction and Mental Health (CAMH) by the WSIB. He attended the program commencing in August, 2009. He was diagnosed at intake as suffering from: PTSD (Chronic, Moderate); Panic Disorder With Agoraphobia; Major Depressive Disorder ("MDD") (Recurrent, Mild to Moderate). His symptoms of depression and anxiety prevented him from returning to work.

I have set out this lengthy history for a couple of reasons. First, it demonstrates clearly that Mr. Georgoudakis was well aware of how the system worked as he collected WSIB benefits on many occasions. I am not suggesting that it was inappropriate for him to have done so. However, if an incident occurred or Mr. Georgoudakis felt that he was suffering mental-health issues brought on by his workplace, he took himself out of the employment setting. I also have no doubt that some of the traumatic events that he witnessed were severe enough to cause him to suffer from PTSD in the late 1990's and early 2000's.

Effective July 12, 2010, the Grievor was approved by CAMH to return to work on modified duties. Ms. Sanda Stojicic was the Disability Management Specialist (DMS) in Occupational Health and Claims Management at the TTC who handled Mr. Georgoudakis's claim file. On June 22, 2010, Ms. Stojicic wrote to Mr. Georgoudakis offering a detailed return-to-work program. Mr. Georgoudakis was offered a position in

the Purchasing and Sales, Materials and Procurement Department. The job entailed making telephone calls to TTC suppliers, following up on shipments, performing data entry and other duties as required. It was a sedentary office position and met the temporary psychological work restrictions set out in the CAMH letter.

Mr. Georgoudakis returned to work in the Materials and Procurement job. The plan was to last for twelve weeks, with graduated hours of work starting at four hours per day twice per week building up to a thirty-five-hour work week by September 20, 2010. On July 15, 2010, at Mr. Georgoudakis's request, his weekly schedule of hours worked in the RTW plan was accelerated, with the result that he would be working a 35-hour work week by August 30, 2010.

On July 27, 2010, CAMH reported that because of his improvements on the job and with continued opportunity for exposure to the subway system, he was anticipated to be able to return to subway duties within four weeks. This time frame was then accelerated even further, as it was agreed that he would return to subway operator duties as early as August 9, 2010. This was almost two months earlier than originally expected. After participating in a nine-day re-certification program and a gradual return to driving the subway, Mr. Georgoudakis resumed full-time duties as an operator with no restrictions on September 26, 2010.

In October, 2010, Mr. Georgoudakis was discharged from the CAMH program with a diagnosis of: Anxiety Disorder Not Otherwise Specified (features of PTSD); Major Depressive Disorder, Recurrent, In Partial Remission; Panic Disorder with Agoraphobia – In Remission. Therefore, although he was not 100% and still suffered from some psychological symptoms, he was found fit to return to work as a subway operator and did so. It was noted that Mr. Georgoudakis's psychological issues related to the workplace incident had, for the most part, resolved and he had reached maximum

psychological recovery. He was psychologically capable of engaging in regular duties on a full-time basis. No psychological work restrictions or precautions were identified.

Mr. Georgoudakis worked as a subway operator from his return to work in August/September, 2010, until the end of June, 2011. He testified that he was not psychologically ready to return to work in the summer of 2010. He also testified that he downplayed his functioning and the frequency of his symptoms to CAMH in order to obtain approval by CAMH to return to work on modified duties as he felt strongly that the program was no longer helping him.

Mr. Georgoudakis testified that he continued to suffer from serious psychological symptoms once he returned to work in the summer of 2010. He testified that the sights, sounds and smells had him feeling constantly uneasy and unstable. He testified that the smell from the brakes and railcars, as well as the noises and squeaking that the cars would make, would bring him back, and would represent flashbacks and remembered painful situations.

Mr. Georgoudakis testified that he was terrified to be back at work and went to the washroom when he had crying spells. He also testified that sometimes he would vomit, either in the washroom or out his cab window in the subway tunnel. He testified that following his return to work he felt his mental health was deteriorating, began crying while at work, and had suicidal thoughts.

In December, 2010, Mr. Georgoudakis testified that he was mentally crashing and needed to rest at home for a period of time. As a result, he testified that he saw Dr. Zachariadis and booked off sick on December 20, 21 and 22, 2010, and then started vacation for a week on December 27, 2010. Mr. Georgoudakis testified that between those days and his off days, he was off work for a period of time and was able to continue working. No medical note was provided for this time period.

Mr. Georgoudakis also testified that he again had a hard time coping because of his symptoms in or around May, 2011, and saw Dr. Zachariadis for help. Dr. Zachariadis reported that Mr. Georgoudakis was seen in his clinic on May 10, 2011, for a stressful situation.

In his May 11, 2011, clinical note, Dr. Zachariadis recorded that Mr. Georgoudakis was under a lot of stress at home and at work, but he does not indicate the cause of the stress or that the workplace stress is causing a reoccurrence of his PTSD.

On May 18, 2011, Dr. Zachariadis filled out an application for sick benefits on behalf of Mr. Georgoudakis. Under the heading "primary diagnosis," he noted "stress-related anxiety state and depression, involving symptoms of crying spells, restless and broken sleep, and withdrawal symptoms." Dr. Zachariadis also provided a doctor's note that stated:

Dr. George P. Zachariadis
855 Broadview Avenue, Suite 101
Toronto, Ontario M4K 3Z1
Tel.: 416-466-6699 / Fax: 416-465-6179

Date: May 18, 2011

To whom it may concern

Re: Mr. Georgoudakis Tony, D.O.B. May 22, 1965

This is to certify that the above mentioned patient was examined in our clinic on May 10, 2011 for a stressful situation. He was reassessed today, May 18, 2011, and was found to be fit to return to his usual work duties as of May 20, 2011.

George P. Zachariadis, M.D.

Again, this note does not indicate the cause of Mr. Georgoudakis's stress. It does not link the mental-health symptoms he was allegedly suffering from at the time to events in the workplace either historically, or in May, 2018.

Mr. Georgoudakis returned to his regular work duties as of May 20, 2011, and no accommodation of any kind was sought. He testified that he suffered memory lapses while working due to flashbacks, from once per day before he went off work in May, 2011, to three or four times a week after he returned to work on May 20, 2011. Mr. Georgoudakis testified that he was afraid that if he did not return to work, he might be sent back to CAMH. It is not clear how he felt that could happen and who could force him into a treatment plan that he did not want.

THE CREDIBILITY OF MR. GEORGOUDAKIS

The Grievor's credibility or lack of it is a key issue in this case. Much of the medical evidence in support of his claim for LTD is based on his self-reporting. Generally speaking, the Grievor was not a good witness. He was evasive, vague, dishonest, argumentative and unresponsive when being cross-examined. There are two specific incidents in, or examples of, his testimony that are of serious concern to me.

The first pertains to a request for an adjournment by the Union. On January 19, 2017, the Union requested an adjournment of a hearing date that had been scheduled for January 20 to enable Mr. Georgoudakis to attend to a medical emergency. The TTC requested substantiation of the medical appointments before it was prepared to share the costs of the cancellation of the hearing date. When the hearing resumed on March 6, 2017, the Grievor testified that he took his son to a medical appointment at 3:30 p.m. on January 20 in Leamington, Ontario, and provided a copy of an appointment confirmation card, which was entered into evidence. He testified that he and his son attended the appointment and that he drove his son to the appointment. When asked if

he attended any other appointments that day, he indicated that he believed there was another appointment at the Windsor hospital for some tests. He was asked if the tests took place in the morning and he indicated that it was either the morning or around 12 or 12:30. He wasn't sure of the exact time.

The employer had Mr. Georgoudakis surveilled on January 20. The surveillance report was introduced as an exhibit and it makes it very clear that Mr. Georgoudakis was not truthful in his testimony. He lied about the doctor's visits and was caught in the lie. After being caught, he provided an explanation that, had he given in the first place, would have been an adequate explanation. But he did not. He chose to give fabricated evidence about medical appointments he knew he did not attend. I am very concerned with regard to the demeanour of Mr. Georgoudakis while offering this false testimony and with his angry reaction when he got caught. He did not want to talk about his son's condition (which is understandable) but after having just been caught in a lie, he stated: "I have nothing more to say about this. This is ridiculous. I don't need this. That is that. I don't want it brought up again. It has nothing to do with my TTC case and I am disgusted that you would stoop to this, but I am not surprised." He clearly felt no remorse for having offered false and fabricated evidence while under oath. In fact, he became aggressive and belligerent.

The second example of the Grievor's testimony that concerns me occurred while he was off work in 2012. On February 17, 2012, Mr. Georgoudakis was involved in a motor vehicle accident. He filed an insurance claim in which he asked for \$1 million for loss of income and loss of earning capacity. He claimed that he could no longer work as a result of the injuries caused in the accident. This claim for lost earnings is for the very same period as he is claiming to be totally disabled and unable to work due to PTSD in this case. Moreover, in the statement of claim for that motor vehicle accident, he did not

indicate that he was already off work as a result of his claimed total disability due to PTSD at the time of the accident. Accordingly, he clearly did not tell the truth in the court pleadings. The claim was settled on December 14, 2015, and according to the settlement, Mr. Georgoudakis received \$40,000 for income replacement and \$10,000 for past and future medical benefits. Mr. Georgoudakis signed off on the release.

The Grievor never disclosed the motor vehicle accident, the court action or the settlement during this hearing until he was confronted with it in cross-examination. It appears that he never advised his counsel that he received any money even though he admitted that it had previously been explained to him that he would have to report the settlement. He claimed to not view it as income. And even when he was initially asked in cross-examination if there was a settlement, he did not admit that he received \$50,000. Instead, he testified that the settlement was only for \$5,000 to \$6,000, of which he received nothing. Even after he was shown the release, he remained evasive, argumentative and unresponsive.

In terms of whether the cheque was written to him or to his lawyer, he initially claimed that it went to his lawyer but then later admitted that the cheque was written to him. His answer changed continuously in terms of how much money his lawyer took for fees and what he received. At one point, he indicated that he only received \$5,000 to \$7,000 as his lawyer took the rest. However, no proof was ever provided that his lawyer took anything. Despite repeated requests that his bank records be produced, nothing ever was.

Again, I have concerns about the Grievor's failure to be honest. Had the employer not discovered the motor vehicle accident insurance claim, it is clear that Mr. Georgoudakis would never have brought it up. It is clear that he has no qualms about being dishonest when it suits his purposes. There are many, many more examples where Mr. Georgoudakis was not forthcoming in his testimony and was caught being untruthful.

Which brings me back to where I started, which is that Mr. Georgoudakis testified at length and in great detail with regard to his mental state and the symptoms he was suffering from when he returned to his subway duties in 2010 until he went off on WSIB benefits in June, 2011.

After his return to work in 2010 until he went off work in June, 2011, Mr. Georgoudakis never told operational management or anyone in Occupational Health and Claims management that he was suffering from a reoccurrence of the mental-health issues he had suffered previously from. During this time period, he never raised any issues with regard to the concerns he appears to have had at the time, with anyone at all in the workplace. Mr. Georgoudakis testified before me in 2016 about events that allegedly occurred in 2010 and 2011. His evidence is very detailed and is set out earlier in this decision. Given the self-serving nature of this testimony, I find myself questioning the existence of and severity of the symptoms he describes. Particularly since he never spoke to anyone at the TTC or claimed a reoccurrence of his previous WSIB claim for PTSD.

Other than a brief doctor's note in May, 2011, describing him as suffering from a "stressful situation" and an application for sick benefits where Dr. Zachariadis noted a diagnosis of "stress-related anxiety state and depression" for which sick leave, as opposed to WSIB benefits, was sought, absolutely nothing was ever reported at the time to his employer. There is nothing in either the doctor's note or the application form that links his stress and depression to the workplace. He has collected WSIB benefits on numerous occasions in the course of his employment so is well aware of how the system works. It also does not appear that he told his doctor the extent of the symptoms he related in his testimony as nothing that severe is documented by Dr. Zachariadis in his clinical notes.

EVENTS BETWEEN JUNE, 2011, AND DECEMBER, 2011

I would now like to turn to the start of the time period that is relevant to the application for and denial of LTD benefits that gave rise to the grievance before me. Again, I heard detailed evidence from Mr. Georgoudakis about the severe mental-health issues he was suffering from at the time relevant to this claim for LTD benefits. I note that many of the clinical notes and doctor's reports merely chronicle or report on what was told to the doctor by Mr. Georgoudakis.

On June 29, 2011, Mr. Georgoudakis went off work for a back injury suffered while working. He filled out an occupational injury report indicating that he "was lifting a train chair or seat to gain access to switches. The chair was jammed and I overexerted my back lifting it." The "emotional trauma" box is not checked off on the form. His only complaint is physical, as he indicated a strain of his middle left back. The Grievor indicated on the form that he was offered alternative work at the time consisting of customer service and pamphlet distribution.

Dr. Zachariadis filled out a Supplemental Medical Information (SMI) form provided to him by the Commission on June 29, 2011. It confirms a primary diagnosis of "upper + middle back strain." The space on the form for a secondary diagnosis is blank. The form also indicated that Mr. Georgoudakis was totally unfit for work and was to be off work until further notice.

On July 7, 2011, a Disability Management Specialist, Mike Stinson, wrote to Mr. Georgoudakis and offered him modified duties again. He was also provided another SMI to have his doctor complete to assess his ability to do modified work and was asked to return it to the Commission by July 25. He never had it completed.

Mr. Georgoudakis received loss of earnings (LOE) benefits from the WSIB from June 29 to July 19, 2011. He testified that in July and August, 2011, his back was very tender.

Dr. Zachariadis filled out a Form 26 for the WSIB on July 29, 2011, and recorded that Mr. Georgoudakis had ongoing pain and stiffness in his upper and lower back, difficulty undressing and bathing, and poor sleep. Between June and August, 2011, Mr. Georgoudakis saw Dr. Zachariadis on six occasions. On each of those visits, Dr. Zachariadis's clinical notes indicate that Mr. Georgoudakis continued to suffer upper and lower back pain and stiffness, soft tissue tenderness and spasm and ongoing upper and lower back myofascial pain. There is no mention of any psychological issues.

On August 10, 2011, Dr. Zachariadis filled out an SMI form that again confirmed that Mr. Georgoudakis was suffering from a back injury and indicated that he could return to a modified job that did not entail any lifting, carrying, pushing or pulling and involved minimal stair climbing. A number of specific limitations are set out on the form.

It is important to note that Dr. Zachariadis answers the question "Are there any psychological conditions (including occupational workplace stressors) that prevent a return to work?" in the negative. He expressly indicates on the SMI form that there are no psychological conditions. Dr. Zachariadis certifies Mr. Georgoudakis as able to return to modified duties with restrictions as of August 15, 2011. On August 16, 2011, Dr. Zachariadis provided further clarification as to Mr. Georgoudakis's restrictions and indicated that he recommended a trial of sedentary modified work for four hours a day.

Mr. Georgoudakis testified that he attempted to return to modified duties on August 18, 2011, but re-injured his back on the way to the place he had to report to. During his commute, he had to stand on the train for 20 minutes, walk for two city blocks, and climb four sets of stairs. He allegedly began suffering from severe pain, dizziness and nausea. He called his wife, who drove him to his doctor on an emergency basis.

In the termination decision, I dealt with events that occurred between this unsuccessful attempt to return to work and December, 2011 and concluded as follows:

On August 23, Ms. Stojicic wrote to Mr. Georgoudakis asking him to have his doctor complete a SMI form and return it to her by September 2, 2011. It was not

returned by September 2. Despite the fact that he was not receiving WSIB benefits, Mr. Georgoudakis made no further attempts to return to work on modified duties. On October 5, Ms. Stojicic contacted the WSIB and left a voice message for Mr. Georgoudakis's case manager indicating that she would like to discuss the file.

On October 14, 2011, Ms. Stojicic spoke to the WSIB and was advised that Mr. Georgoudakis had been sent to the WSIB's Regional Evaluation Centre ("REC") for an assessment on September 28, 2011. An examination at the REC is conducted by a multidisciplinary team of specialists who assess an employee's condition and provide recommendations. In the initial report dated September 28, it was noted that Mr. Georgoudakis should continue off work and receive a magnetic resonance imaging (MRI) scan. The MRI took place on October 13, 2011, and was found to be unremarkable. It was recommended that after three to four weeks of physiotherapy and a self-directed exercise program, he would be fit to return to his regular duties.

When Ms. Stojicic spoke to the WSIB on October 14, she was told that it was the position of the WSIB that the Grievor was only partly disabled and was therefore able to go on a transitional work program ("TWP") on modified duties. Ms. Stojicic confirmed that the TTC had offered suitable work to the Grievor and that he had failed to return to work. Ms. Stojicic suggested that the WSIB assign a return-to-work specialist to the case and that this person attend a return-to-work meeting at the TTC with the Grievor and his Union. His case manager agreed.

On October 25, 2011, the meeting was held and present was a representative of the union, the Grievor, Ms. Stojicic and a WSIB return-to-work specialist. At the meeting, the WSIB representative indicated that the REC examination had confirmed that there was no objective medical to support total disability and that the Grievor should be able to do modified duties. He could start back at work commencing the week of October 30. At the meeting, the group discussed a variety of modified duties (customer service, crash gates, sedentary office) that were available and consistent with the REC examination and were consistent with his attending physician's recent medical dated October 8, 2011.

The Grievor refused to sign off on the return-to-work plan and indicated that he was not fit to return to work. He indicated that he wanted to have the REC assessment reviewed by Dr. Zachariadis and to see a specialist. It is important to note that there was no mention of any psychiatric impairment at this meeting. Ms. Stojicic asked Mr. Georgoudakis to get back to her before Friday, October 28, and that she would assume he could return to work on Monday, October 31, if she did not hear from him. On October 28, Mr. Georgoudakis left a message for

Ms. Stojicic indicating that he was seeing his doctor on October 31 and would not be coming in to work.

The WSIB wrote to Mr. Georgoudakis on November 8, 2011, summarizing what had occurred to date and updating him on the status of his claim. The letter outlined the results of the REC assessment and noted that the results of the MRI on the Grievor's thoracic and lumbar spine that had been conducted were unremarkable and there was no objective evidence of any abnormality. He was advised that the WSIB considered him fit to return to modified duties and that after four weeks of physiotherapy, he could resume full duties and his pre-injury job by December 12, 2011.

Therefore, at this point, there had been four medical confirmations that the Grievor was fit to return to work and perform modified duties. It had been confirmed: by Dr. Zachariadis in the August 10, 2011, SMI form; by Dr. Zachariadis in the August 17 discussion he had with Ms. Stojicic; by Dr. Zachariadis in the answers to the questions asked by the Commission in the letter dated August 12 and completed by him on August 16; and by the REC assessment conducted by the WSIB which included an MRI indicating that there was no objective evidence of any abnormality in his spine, that there was no objective medical to support total disability and that the Grievor could do modified duties.

Throughout this entire period, Mr. Georgoudakis claimed in his testimony before me that he was suffering from severe PTSD and other mental disabilities. The medical reports submitted to the TTC by Dr. Zachariadis do not reflect this. In fact, mental-health issues are not mentioned at all in Dr. Zachariadis's reports. It also appears that Mr. Georgoudakis never told Ms. Stojicic or anyone else at the TTC about these issues.

EVENTS BETWEEN DECEMBER, 2011 AND APRIL, 2012

On December 7, 2011, Dr. Zachariadis wrote the following:

To Whom It May Concern

Re: Mr. Georgoudakis Tony, D.O.B. May 22, 1965

This is to confirm that the above noted patient has been under my medical care since June 21, 2011. He was diagnosed with the following medical conditions.

1. Major Depression

2. Panic Attacks
3. Chronic Anxiety Disorder
4. Post-Traumatic Stress Disorder
5. Chronic Sleep Disorder

The above medical conditions were due to a stressful job situation. He was strongly advised to stay off work until further notice. His appointment for psychiatric evaluation is still pending.

George P. Zachariadis, M.D.

This note was not received by the TTC until January 24, 2012. This was the first time that Dr. Zachariadis suggested in a note or report that the Grievor was suffering from psychiatric issues and that they relate to a “stressful job situation”. Even though Mr. Georgoudakis has been off work for almost six months at this point, there is no indication of what triggered these medical conditions.

On March 12, 2012, the TTC received an updated SMI form which was dated March 10, 2012, and had been completed by Dr. Zachariadis on that date. Under the heading primary diagnosis it states, “Ongoing PTSD, MDD/Panic attacks (resulting from 21/02/2007 incident)”. No explanation of any triggering event is provided and Mr. Georgoudakis at this point had not worked in subway operations for almost nine months. Dr. Zachariadis indicated on the SMI form that “a trial period of working in a modified job with no exposure to the stressors that caused his PTSD can be initiated at any time.” In response to specific questions on the form, Dr. Zachariadis indicated that Mr. Georgoudakis could undergo recommended therapies and “work in a modified job away from the stressors that caused/triggered his PTSD” and again later “a trial period of working in a modified job (not exposed to the stressors mentioned above) can be initiated at any time.” The SMI makes it very clear that in the opinion of Dr. Zachariadis,

Mr. Georgoudakis can return to modified duties (as long as they do not entail working in a subway) as he has completely recovered from the injuries suffered in June, 2011.

In his reply evidence, Mr. Georgoudakis testified that in the period January to April, 2012, the TTC never offered him light duties and that he would have participated in a temporary work program performing modified duties as long as the work met his restrictions. He indicated that he was “on board” for attempting any alternative work that respected his restrictions. He testified that he let Occupational Health and Claims Management know this but that the TTC insisted that he return to his position as a subway operator.

This testimony is not at all consistent with the evidence he gave in chief with regard to the severity of the symptoms he was suffering at this time. He testified in chief that he was totally disabled in this period. On February 10, 2012, Mr. Georgoudakis reported to Dr. Zachariadis that he continued to be depressed/anxious, and was suffering from PTSD, frequent panic attacks and fragmented sleep pattern.

Mr. Georgoudakis met with Dr. S.H-Dief, a psychiatrist, on March 6, 2012. He testified in chief that at this time he told Dr. Dief about the frequency of his flashbacks (from one or two per day to five in the course of a few hours). He testified that the content of these flashbacks consisted of traumatizing moments where people tried to hurt or kill themselves while he was driving the train, witnessing assaults on other individuals, and visions of people dead or hurt that he saw while at work. Mr. Georgoudakis also testified that he told Dr. Dief about the frequency of his nightmares (at least once per night). He testified as to the content of his nightmares, consisting of seeing the people who he has seen dead talking to him. He testified that he told Dr. Dief about his flashbacks and nightmares, as well as about how he chewed and bit his fingers, scratched and cut his arms, and suffered from night terrors. Based on all of this self-reported symptoms, Dr Dief concluded that Mr. Georgoudakis suffered from severe PTSD.

The testimony provided by Mr. Georgoudakis in chief and in reply is at best inconsistent but at worst completely fabricated. I reject his testimony that he was “on board” for attempting any alternative work that respected his restrictions but that the TTC insisted that he return to his position as a subway operator. As much as I would like to believe Mr. Georgoudakis, this testimony makes no sense. I have absolutely no reason to believe that the TTC would reject Dr. Zachariadis’s medical opinion. Why would they? They had made repeated attempts to return Mr. Georgoudakis to modified work and he had continuously rejected these efforts. In addition, he was well aware of the return to work process and the type of modified duties that were available as he had participated in the process as recently as 2010.

I was provided with some business records routinely kept by the TTC. Although she did not testify. I was given the clinical records from Occupational Health and Claims Management kept by Ms. Stojicic. She kept detailed notes of her ongoing interactions with Mr. Georgoudakis. On November 3, 2011, she notes that she reminded Mr. Georgoudakis that the TTC had modified duties available and that she would do her best to accommodate him when he was ready to return to work. She asked him to advise her when he would be reporting to work and indicated that she left it up to the employee to notify the TTC when he would be returning to work.

On December 8, 2011, she notes that she wrote to the Grievor inquiring into his status. She notes on January 17, 2012, that she had not had a response from the Grievor. On January 24, 2012, the notes of Ms. Stojicic indicate that the Grievor called her to ask some questions and inquire as to whether the TTC had received the note from Dr. Zachariadis dated December 7, 2011. She told him that they had not and he indicated he would send it. Ms. Stojicic’s note also indicated the Grievor told her he was totally disabled. On February 1, 2012, she wrote to Mr. Georgoudakis and requested that he get Dr. Zachariadis to fill out an updated SMI. This is the one dated March 10 and received by the TTC on March 12, 2012.

I do not believe that Mr. Georgoudakis was interested in or willing to return to modified duties at this time. As I noted, his evidence is very inconsistent as he claimed while testifying in chief to be totally disabled during this period.

THE MEDICAL EVIDENCE GENERALLY

The medical evidence before me in this case overlaps significantly with the medical evidence called in the termination case. The overlapping evidence includes clinical notes and various reports filled out by Dr. Zachariadis and a medical report prepared by Dr. Dief. Neither Dr. Zachariadis nor Dr. Dief testified in either proceeding. Unfortunately Dr. Zachariadis passed away prior to this hearing. Dr. Dief was not called as a witness.

In the decision in the termination case, I came to the following conclusions:

I have reviewed the report submitted [prepared by Dr Dief] and I do not find it to be of much assistance. It is not clear when Dr. Dief actually saw Mr. Georgoudakis. It references an initial assessment on December 30, 2011, but does not indicate if Dr. Dief saw him again around the time that the report was written or only on the one occasion. The report sets out a narrative that was clearly provided to Dr. Dief by the Grievor. The concluding paragraph starts with “As you know this 46-year-old male suffers from severe Post-Traumatic Stress Disorder.” It is not clear to me that Dr. Dief medically examined the Grievor and diagnosed PTSD, or again, as Dr. Menchions did, is merely reporting what he was told by the Grievor. Dr. Dief says “as you know”, but Dr. Zachariadis had not concluded that Mr. Georgoudakis suffered from severe PTSD but from moderate PTSD symptoms.

The report does not indicate whether the Grievor could perform his regular duties or any form of modified duties. It does not indicate whether he is totally or partly disabled from working. This report is not consistent with the report provided to the commission by Dr. Zachariadis which clearly indicated the Grievor suffered from moderate PTSD symptoms and could return to work on modified duties. Even though Dr. Dief is a psychiatrist, given the vagueness and inadequacies of his report, I prefer the assessment of the Grievor’s capabilities given by Dr.

Zachariadis over that of Dr. Dief. The fact that Dr. Zachariadis was in regular contact with the Grievor and had seen him throughout the time he had been off work puts him in a much better position to assess Mr. Georgoudakis.

Dr. Dief did not testify in either the termination proceedings or in the current one. I do not therefore have the benefit of receiving any explanation from him as to how he came to the conclusions he did, nor was he subjected to cross-examination. In my view, there is no reason to change the analysis or the conclusions that I came to with regard to the report prepared by Dr. Dief.

Dr. Stanley Dermer was also called by the Union in this case. He was retained by the Union a year after Mr. Georgoudakis's termination pursuant to the discharge case, to conduct an Independent Medical Examination (an IME) and did so. He prepared a report, the admissibility of which was argued before me in the discharge case, and in a decision dated July 5, 2016, I admitted the report, stating:

At this point in the proceedings, I cannot determine how helpful the report will be to the determination that I must make. Nor can I assess the weight, if any, that the report will ultimately be given and obviously that is a matter that the parties can make argument on. The preliminary motion to exclude the medical report prepared by Dr. Dermer is therefore dismissed. The report is relevant and admissible.

The Union did not rely on Dr. Dermer's report or call him as a witness in the termination proceedings. The report did not therefore play any role in the termination decision. In this case, Dr. Dermer commenced his evidence in chief but due to health issues was unable to complete his evidence. He was not available for cross-examination.

Counsel for the employer took the position that as Dr. Dermer was not able to complete his testimony in this case and the TTC did not have the opportunity to cross-examine him that Dr. Dermer's report should no longer be considered admissible and should be given no weight at all. Had the TTC known in advance that Dr. Dermer would be unavailable for cross-examination, it would have objected to the admissibility of the

report on that basis. Given that the inability to cross-examination did not arise until later, the TTC could not make such an argument.

Accordingly, the TTC relied upon the jurisprudence set out below for the proposition that a medical report ought not be admissible if a party does not have the right to cross-examine the author. It was suggested that it would be a complete denial of natural justice to place any weight on Dr. Dermer's report or his oral evidence in direct examination. Cross-examination of a witness is a fundamental litigation right and it ensures justice in every case. Under the adversarial system of arbitration, cross-examination is the key to discovering the truth. The Supreme Court of Canada articulated the vital role that cross-examinations play in making a full answer and defence in *R v Osolin* [1993] S.C.J. 595, 109 D.L.R (4th) 478:

There can be no question of the importance of cross-examination. It is of essential importance in determining whether a witness is credible. Even with the most honest witness cross-examination can provide the means to explore the frailties of the testimony. For example, it can demonstrate a witness' weakness of sight or hearing. It can establish that the existing weather conditions may have limited the ability of a witness to observe, or that medication taken by the witness would have distorted vision or hearing. Its importance cannot be denied. It is the ultimate means of demonstrating truth and of testing veracity.

Employer counsel pointed out that this principle is well established in the arbitral jurisprudence. In a case very similar to ours, *I.A.M. & A.W. v Air Canada* [1998] 51 C.L.A.S. 107, the employer attempted to introduce medical documentation into evidence without calling the authors for cross-examination. Arbitrator Dissanayake addresses this issue by quoting largely from a decision of Arbitrator Mitchnick in *Miracle Food Mart of Canada* in which he reviews both the arbitral and court jurisprudence. Of importance, Arbitrator Mitchnick states at page 8:

I do not disagree with any of the above formulation. Indeed, in the present case, with the medical reports so subjective and germane to a central issue in the case, there is no possibility that I in the exercise of my discretion would admit

them into evidence without the requested undertaking by the employer to have the doctors appear on the witness stand for cross examination.

Counsel suggested that our case is indistinguishable and therefore Dr. Dermer's report should not be considered admissible evidence and neither his report nor his oral testimony should be given any weight.

Union counsel argued that the case law indicates that where a witness becomes incapacitated and is unable to complete his evidence in chief or under cross-examination, the evidence is not struck but the issue is the weight to be given the evidence (*Ontario Power Generation and Society of Energy Professionals*, (2015) 264 L.A.C. (4th) 283 (Surdykowski) at para. 33). Under Section 52 of the Evidence Act, medical records and reports are admissible as an exception to the hearsay rule. However, the opposing party may require the author of those records to be produced for cross-examination. In this case, Dr. Dermer was, due to illness, unable to be available for cross-examination.

Counsel for the Union also suggested that Dr. Dermer's medical report is admissible under the criteria of necessity and reliability (*R. v. Khan*, [1990] 2 S.C.R. 531). They are admissible under the criteria of necessity as, due to the onset of his illness, he was unable to complete his testimony. His report should be deemed to be reliable, given his qualifications as a professional psychiatrist, as well as the fact that he has been previously characterized as an experienced expert in the field.

I would note that there was no dispute between the parties with regard to the admissibility of the reports prepared by Dr. Zachariadis and his clinical notes. It seems somewhat inconsistent to me to agree to admit Dr. Zachariadis's reports and not the one prepared by Dr. Dermer. Accordingly I agree with the submissions of counsel for the Union that in the circumstances of Dr. Dermer's incapacity, the report is admissible. However having said that, I also agree with employer counsel in terms of the weight to

be given to his report. I am not prepared to give the evidence given, nor the report prepared by Dr. Dermer, any weight. The report is very subjective in nature, clearly relies heavily on self-serving information provided by Mr. Georgoudakis and was based on examinations of the Grievor that took place in October, 2014, which is well after the period of time for which LTD benefits are sought.

I would now like to turn to the evidence given by Dr. Peter Findlay. Dr Findlay is another family physician seen by Mr. Georgoudakis during the relevant time period and was called as a witness by the Union. Dr. Findlay started treating Mr. Georgoudakis in 1996 and saw him for the last time in February, 2016. In the time period at issue in this case, December, 2011, to December, 2013, Mr. Georgoudakis was utilizing the services of two family physicians. Dr. Findlay was unaware that Mr. Georgoudakis was seeing Dr. Zachariadis and agreed with employer counsel that it was unusual and atypical for someone to have two family physicians.

In his testimony, Dr. Findlay made it clear on numerous occasions that he accepted the Grievor's self-reporting of symptoms without any objective verification. He admitted that he believed what the Grievor told him. He also admitted that he took no steps to objectively verify what the Grievor was telling him.

Even though the Grievor was seeing Dr. Findlay while he was off work from 2007 to 2010, the Grievor never advised Dr. Findlay that he was not working, nor did he advise him that he was in the PTP program at CAMH. When questioned about this, Dr. Findlay agreed that the Grievor was not giving him the "full picture" of his situation. This is simply another example of the Grievor not being truthful or forthcoming to Dr. Findlay and in general.

The Grievor saw Dr. Findlay on August 10, 2010, for an annual physical examination and then he did not see him again until one year later in May of 2011. At no time in

these two visits did he mention any mental-health issues related to work or otherwise, his complaints were only physical.

He does not see Dr. Findlay again until March 12, 2012, with regard to his motor vehicle accident in February of 2012. At this appointment, he does not tell Dr. Findlay that he has been off work since June 29, 2011, and he does not tell Dr. Findlay that he is claiming to be totally disabled from work due to PTSD and depression. Dr. Findlay testified that based on what the Grievor was reporting to him, he had no idea that he was off work and he had no idea that he was claiming to be totally disabled. The Grievor never even mentioned that he had any mental-health issues to him until May 10, 2013. Clearly, Mr. Georgoudakis was doctor shopping and providing different information to the two family doctors he was seeing at the same time.

On March 21, 2012, Ms. Stojicic wrote to Dr. Bruce Menchions, a medical consultant under contract with the Commission, and asked him to meet with Mr. Georgoudakis and assess whether or not “there is a psychological condition that is disabling him from returning to work in any capacity (including TWP). In your opinion what is the psychological condition? Do you recommend an IME or any other suggestions for further management of this claim?” Dr. Menchions testified in the termination proceedings but not in the LTD case.

In the termination decision, I concluded:

In his testimony Dr. Menchions made it clear that it is not his role to diagnose the individuals referred to him by the Occupational Health and Claims Management Division and that he is not involved in the direct care of Commission employees.

He does not treat Commission employees. It is his role to provide consultative support and advice to the people who work in the Occupational Health and Claims Management Division. Dr. Menchions did not answer any of the questions posed by Ms. Stojicic and the brief notes he wrote of his meeting with Mr.

Georgoudakis on March 21, 2012, primarily record statements made to him by Mr. Georgoudakis.

...

Counsel for the union in his final submissions argued that Dr. Menchions applied his medical expertise to assess Mr. Georgoudakis and concluded that he was totally disabled. I have carefully reviewed the evidence given by Dr. Menchions and conclude: that he did not provide a medical opinion; that he recorded what the Grievor self-reported to him; and that he was not in a position to corroborate what he was told by Mr. Georgoudakis. As he put it, "I just reported what I was told and I did not form my own opinion. I avoided doing that". In answer to the question from Commission counsel "Was it your opinion that he was totally disabled?", Dr. Menchions responded: "I have no medical opinion. I can only report what Mr. Georgoudakis told me and cannot give it any objective weight. I am just like a lay person reporting what I have been told by Mr. Georgoudakis". Based on this testimony, I do not find the evidence of Dr. Menchions to be helpful and am not prepared to give it any weight.

In the present case, Dr Menchions did not testify. Therefore, in my view, there is no reason to change the analysis or the conclusions that I came to with regard to the testimony given by Dr. Menchions.

THE MANULIFE ASSESSMENT OF THE CLAIM FOR LTD

The TTC called Dr. Diana Blank as a witness. Dr. Blank is a medical doctor with a specialist in psychiatry. She has worked for a number of insurance companies. In her positions for these companies, she has assessed and managed disability claims, either STD or LTD and rendered decisions on claims. She has also worked providing psychiatric care to medically ill patients. She worked for North Halton Medical Clinic, where she provided general psychiatric care to their patient population covering all types of diagnosis.

Dr. Blank has been a medical consultant for Manulife since 2013. She is not an employee of Manulife. In her role as a consultant, she does not assess patients. Her

role is only to review the documentation and to provide an opinion based on the documentation. She testified that in this role, she sees herself as an advocate for the claimant insofar as she provides a specialized understanding of the medical documentation and can advise what additional information would be of assistance.

Dr. Blank received the referral for Mr. Georgoudakis and conducted the review on June 17, 2015. The purpose of the review was to assess the medical documentation on file and to provide an opinion with respect to whether there was sufficient medical information to substantiate functional impairment preventing the Grievor from returning to work.

Based on the documentation from 2007 to 2010, Dr. Blank stated in her report that the Grievor was diagnosed with chronic PTSD recurrent and that he had received treatment from CAMH PTP until August, 2010. At the end of the treatment, the Grievor was found to be psychologically capable of returning to work full-time with no restrictions. The discharge diagnosis was not PTSD, it was anxiety disorder not otherwise specified with features of PTSD. This meant that the Grievor had some PTSD symptoms but did not have full-blown PTSD. Dr. Blank noted that the Grievor remained at work in his regular position as a subway operator without limitations until he commenced a leave of absence on June 29, 2011, as the result of a back injury.

With respect to Dr. Zachariadis's December 7, 2011, report, the evidence of Dr. Blank was that it was not a helpful report because: it does not indicate when the Grievor was diagnosed, whether this is a chronic presentation, or whether the assessment is new or from 2007; it provided no description of the severity of the symptoms and how they impacted the Grievor's functioning; and there was no description of how these symptoms were different from when the Grievor previously returned to work.

With respect to Dr. Dief's letter dated March 6, 2012, Dr. Blank's evidence was that the report was brief compared to reports she would expect and that there were a lot of

components to the report that were missing, especially the history of presenting illness – which is when a doctor describes what’s going on, including the symptoms and the disability. Dr. Blank also testified that some of the symptoms cited by Dr. Dief can be symptoms of PTSD, but that there were not enough present to meet the criteria for a diagnosis of PTSD.

With respect to the Supplemental Medical Information form filled out by Dr. Zachariadis, dated March 10, 2012, Dr. Blank’s evidence was that Dr. Zachariadis described PTSD as starting around 2003, eight years prior to 2011. The report does not indicated the severity or frequency of the symptoms and their impact on the Grievor’s functioning.

Dr. Blank concluded that the medical documentation failed to establish that the Grievor suffered from severe, consistent psychological impairment. It did not show that the symptoms met the diagnosis for severe PTSD and did not demonstrate functional impairment preventing the Grievor from engaging in work. There was no information to show what triggered or caused the symptoms to become so severe in 2011 and why they remained so severe for four years. She noted that a critical deficiency in the Grievor’s claim was that he was able to return to work as a subway operator following his discharge from the CAMH PTP program for almost one year and that there was never any evidence provided of the reason for his PTSD to be triggered in December, 2011, after he had been away from the subway and his duties for almost six months.

She testified that she was therefore unclear how his presentation overall had been impaired from working and how much more severe his impairment was compared to 2010 when he went back to work. Dr. Blank had concerns that the documentation on file did not provide treatment specifics and that symptoms were not described in terms of severity and frequency. Based on the pharmacological record, Dr. Blank confirmed that the Grievor’s treatment was also very spotty.

I found Dr. Blank to be a credible witness. Her evidence was provided in a fair and reasoned manner. Although she is a medical consultant retained by Manulife, her

analysis of the medical evidence was very matter-of-fact and objective. She did not come across as an advocate for Manulife.

The Grievor did not apply for LTD benefits until December 28, 2012, several months after the time period for applying for LTD had expired. The TTC facilitated his application by agreeing to a significant time extension. There were also several other occasions during the adjudication of the claim where the TTC made special requests to Manulife that were to Mr. Georgoudakis's advantage and approved timeline extensions to ensure that the Grievor's claim was adjudicated fully and fairly. I am satisfied that the TTC did everything it could to assist the Grievor in respect of his LTD claim. For example, Manulife denied the claim in a letter to the Grievor on February 12, 2013. On August 12, 2013, the TTC submitted further medical information to Manulife in support of the Grievor's claim. Specifically, the TTC submitted Dr. Dief's report and Dr. Zachariadis's SMI dated March 10, 2012. Manulife reviewed the file again as a result of the TTC's request, but it still denied the claim.

It is important to note that at all times during the adjudication of the Grievor's claim by Manulife he was assumed to be telling the truth and his credibility was never questioned. I am satisfied that the Grievor was provided with procedural fairness in the manner in which his claim was handled by Manulife and that the TTC did all it could to support and facilitate his claim for LTD.

I am not bound in any way by the findings reached by Manulife with regard to the claim for LTD made by Mr. Georgoudakis. It is not my function to conduct a hearing into the fairness of the conclusions. Rather it is my role to make a determination, based on the evidence presented to me, as to whether or not the Grievor was entitled to LTD from December, 2011, to December, 2013. The Grievor testified at length about how the TTC had ruined his life, failed to support him and was a terrible employer. I do not agree that

the evidence in this case supports that conclusion and wanted to include in this decision that fact that the TTC were supportive of him as demonstrated by the above.

THE LETTER OF UNDERSTANDING REGARDING LTD BENEFITS

As noted earlier in this award, the Collective Agreement provides in Article 1, Section 18 that the TTC agrees to provide all employees on Long Term Disability benefits in accordance with the Letter of Understanding (LOU) dated September 23, 1987. The LOU provides that in the event a dispute arises with respect to eligibility for long-term disability benefit or the amount of such benefit, the matter may be dealt with pursuant to the grievance and arbitration provisions of the Collective Agreement for the purpose of determining such eligibility or amount.

The LOU is lengthy so I will only set out the relevant portions of it. Under the heading Eligibility Requirements, it notes that to be eligible to receive benefits a person must complete the qualifying period. The qualifying period is defined as 26 weeks of sick benefits, plus the exhaustion of Unemployment Insurance Commission (U.T.C.) benefits (if any), plus the exhaustion of all vacation pay entitlement (if any). Under the heading Payment of Benefit, Section 5, it provides:

Upon receipt of proof following the Qualifying Period that the employee became and remained totally and continuously disabled for the Qualifying Period, the benefit will be paid monthly in arrears on the last day of each month.

And Total Disability and Totally Disabled is defined in Section 6 as:

...for the Qualifying Period and for the first 24 months immediately following the Qualifying Period, the employee is wholly and continuously disabled by illness or accidental bodily injury which prevents him or her performing any and every duty of the employee's normal occupation.

The MOU goes on in Section 7 to provide:

Employees in receipt of long term disability benefits must accept any position or work offered by the Commission which is compatible with their disability as determined by the commission's Medical Director and, if disputed, subject to arbitration pursuant to Article I, section 18 of the Collective Agreement between Local 113, Amalgamated Transit Union, and the Commission.

Under the heading Exceptions and Limitations, Section 10 it states that no payment will be made for claims resulting from:

- (a) a disability for which the employee is not under continuing medical supervision and treatment considered satisfactory by the Commission's Medical Examiner and, if disputed, subject to arbitration pursuant to Article I, Section 18 of the Collective Agreement,

And finally Section 11 provides that long term disability benefits shall cease on the earliest of the following events:

- (e) when the employee is not under continuing medical supervision and treatment,

THE UNION'S SUBMISSIONS

The issue in this case is the entitlement of the Grievor to long-term disability benefits from December 30, 2011, to December 30, 2013, during his "own occupation" period. Under the terms of the LTD letter of understanding, he must also show that he was totally disabled during the qualifying period (June 29, 2011, to December 29, 2011) and then was totally disabled for the 24 months thereafter. The Union's position is that the Grievor is entitled to long-term disability (LTD) benefits from December, 2011, to December, 2013, pursuant to the September, 1987, LOU between the parties.

Union counsel argued that the medical evidence, taken as a whole, supports that on the balance of probabilities the Grievor was wholly and continuously disabled from his own occupation during the relevant period. None of the Grievor's treating physicians between 2011 and 2013 were of the medical opinion that he could perform the essential duties of a subway operator and/or could perform that work to the standard of a

reasonable employee. Counsel suggested that the fact that self-reported symptoms form part of the basis of the opinions of the medical professionals who assessed Mr. Georgoudakis does not undermine the reliability of their conclusions. Similarly, even if there may be some degree of exaggeration on the part of the Grievor, the evidence may still be sufficient to establish his disabilities and his entitlement to LTD benefits (see in this regard *Toronto Transit Commission v. Amalgamated Transit Union, Local 113 (Adams Grievance)*, [1997] O.L.A.A. No. 1049 (Saltman) at para. 92).

Union counsel next addressed Section 11(e) of the LOU that provides for the cessation of LTD benefits when the employee is not under continuing medical supervision and treatment. Counsel suggested that it is not the case that the Grievor wasn't under continuing medical supervision and treatment and that I had the jurisdiction to consider the satisfactoriness of the Grievor's treatment under Section 10(a) of the LOU.

The evidence shows that from December 30, 2011, until December 30, 2013, Mr. Georgoudakis attended medical appointments related to his mental-health condition with Dr. Zachariadis, Dr. Findlay and Dr. Dief. He was also seen by the TTC's psychiatric consultant, Dr. Menchions. Dr. Menchions's medical consultant review does not state any concerns with the appropriateness of Mr. Georgoudakis's medical treatment or supervision as of March, 2012.

None of the medical evidence from the medical professionals who saw Mr. Georgoudakis contains any concerns about the satisfactoriness of his medical supervision and treatment. In fact, Dr. Zachariadis confirmed that Mr. Georgoudakis was following the recommended treatment program.

Further, during this period of time, there were several referrals made to specialists who could provide more specialized treatment of Mr. Georgoudakis's mental-health conditions. Dr. Zachariadis referred Mr. Georgoudakis to Dr. Dief in or around December, 2011, then to the Mount Sinai inpatient program in or around the spring of 2012, then to the Mount Sinai outpatient program in or around September, 2012.

In March, 2012, December, 2012, and November, 2013, Dr. Zachariadis also recommended resumption of Mr. Georgoudakis's treatment with Dr. Mitsopoulos, but the TTC, Manulife and the WSIB would not cover the cost of these sessions. In May, 2013, Dr. Findlay also referred Mr. Georgoudakis to Credit Valley Mental Health.

Union counsel pointed out that Dr. Blank testified that referral to multidisciplinary or interdisciplinary programs for PTSD may take six to eight months or a year, depending on when the referral occurred. Given these wait times, along with the cancellation of the Mount Sinai inpatient program, it is not surprising that these referrals did not result in Mr. Georgoudakis being seen at either Mount Sinai or Credit Valley Mental Health during the relevant time period. In the interim, he continued to follow up with Dr. Zachariadis.

When assessing whether the Grievor was under satisfactory continuing medical treatment and supervision, counsel asserted that the case law indicates that long periods between visits to a doctor do not mean that the Grievor's treatment was not satisfactory.

In one case, the arbitration board found that because of the nature of the Grievor's condition, there was no urgent need for more frequent visits and her condition did not require more regular attention by a doctor than she actually received. There was no suggestion that more regular attendance by a doctor would have improved her condition to any meaningful degree (*Steinberg's Ltd. v. R.C.I.U., Local 486* (1979), 23 L.A.C. (2d) 331 (O'Shea)) at para. 24.

As in that case, there is no evidence that more frequent attendance at Dr. Zachariadis's, Dr. Findlay's, or Dr. Dief's offices would have improved the Grievor's condition. Rather, specialized treatment was needed and there were consistent attempts to obtain that treatment by his treating physicians. The fact that there were wait lists and closed

programs does not mean that the Grievor was not receiving satisfactory continuing medical treatment and supervision.

In conclusion, counsel argued that should I accept the argument that Mr. Georgoudakis meets the criteria for total disability, there is no basis upon which his entitlement should be terminated under Section 11(e) of the LOU.

Counsel for the Union next turned to address Section 7 of the LOU, which, in her opinion, is clear and unequivocal and provides that employees in receipt of LTD benefits must accept any position or work offered by the TTC that is compatible with their disability as determined by the TTC's medical director.

It was suggested that no evidence was introduced that the TTC's medical director or medical consultant found any modified work compatible with the Grievor's disability. At most, I have Dr. Menchions's two medical consultant reviews, neither of which make any comment on Mr. Georgoudakis's ability to perform modified work. As such, the strict language of the LOU means that there is no evidence that Mr. Georgoudakis failed to comply with the requirement that he accept any position or work offered by the TTC that was compatible with his disability as determined by the TTC's medical director.

Further, counsel suggested that no evidence has been introduced that Mr. Georgoudakis was offered modified work at any point between December, 2011, and December, 2013. The Union did not concede that any weight could be placed on the clinical records from Occupational Health and Claims Management, but if I do find that they are admissible as business records, the entry on May 4, 2012, from Diane Chyn, the DMS who took over Mr. Georgoudakis's file from Ms. Stojicic, states that no return to work has been developed yet as Occupational Health and Claims Management needs clarification on the employee's psychiatric condition.

As such, the union argued that there is no basis for finding that the Grievor failed to accept any position or work offered by the TTC that was compatible with his disability as determined by the TTC's medical director. Mr. Georgoudakis was totally disabled from

his own occupation, as he was unable to substantially perform all of the duties of his regular occupation. As previously described, Dr. Zachariadis, Dr. Findlay, and Dr. Dermer have all indicated that he could not be employed driving a subway train during his “own occupation” period.

Union counsel pointed out that contrary to the evidence of Dr. Blank, there is no need to show that there was some change in Mr. Georgoudakis’s medical condition. Instead, what needs to be proved on a balance of probabilities is that he was totally disabled as a result of an illness during a particular time frame (*Toronto (City) v. Toronto Civic Employees’ Union, Local 416 (Theodoris Grievance)*, [2010] O.L.A.A. No. 104 (Schmidt) at para. 54). Counsel argued that the Grievor’s testimony, as corroborated by his doctors’ medical reporting, confirms he was totally disabled from his regular occupation as a subway driver during the relevant time frame (June, 2011, to December, 2013).

Total disability does not require that the insured be completely incapable of any activity, only that his condition be such that ordinary and reasonable prudence dictate that he should not engage in a given work activity (*Toronto Transit Commission v. Amalgamated Transit Union, Local 113 (Adams Grievance)*, [1997] O.L.A.A. No. 1049 (Saltman) at para. 80). A grievor is not disentitled to benefits in circumstances wherein his work would cause severe deterioration in his condition notwithstanding that he could perform it (*St. John’s Rehabilitation Hospital v. Ontario Nurses’ Assn. (Greening Grievance)*, [2016] O.L.A.A. No. 155 (McNamee) at para. 88).

Counsel argued that the following evidence shows that reasonable prudence dictated that Mr. Georgoudakis should not work as a subway operator during his “own occupation” period and that performing that work would cause severe deterioration in his condition:

- Mr. Georgoudakis’s testimony was that returning to work full-time as a subway operator between approximately October, 2010, and June, 2011, aggravated his

psychological condition. The symptoms he described, including flashbacks, crying, and vomiting, are consistent with the conclusion that his subway operation duties did cause and would continue to cause severe deterioration in his condition.

- This is corroborated by his absences from work due to illness in December 2010 and May 2011.
- The evidence of Dr. Blank was that the grievor's ongoing risk for re-exposure to traumatic events as a subway operator was high and could lead to relapses in his symptoms in the future.
- Dr. Zachariadis's clinical notes and reports indicate that he advised Mr. Georgoudakis to stay off work until further notice in December 2011 and that, as of March 2012, Mr. Georgoudakis could only work in a modified job away from the stressors that caused/triggered his PTSD and where he was not exposed to those stressors.
- Dr. Dermer's IME stated that, given the severity and chronicity of Mr. Georgoudakis' symptoms and dysfunction, his prognosis for a successful return to work as a subway operator remained guarded.

In terms of the qualifying period, the Union submitted that the evidence confirms that the Grievor satisfies the criteria of showing total disability during the qualifying period, as set out in Section 10 of Appendix "A".

Mr. Georgoudakis testified that he was unable to work as a subway operator because of his back condition until at least November, 2011. This is corroborated by the WSIB Regional Evaluation Centre assessment, which recommended that he continue off work as of September 28, 2011, and that he should be fit to return to his regular duties three to four weeks after October 19, 2011 (i.e. on or around mid-November, 2011). In particular, his inability to work as a subway operator due to his back was a result of his limitations in sitting (a major requirement of the subway operator position), was noted by Dr. Zachariadis.

Mr. Georgoudakis testified that his back injury aggravated a lot of his mental symptoms, but also that staying off work kept him safe due to the risks of being impaired by his mental disability. By November 3, 2011, his psychological symptoms were severe enough that he went to see Dr. Zachariadis, who provided a notation of PTSD. By December 7, 2011, Dr. Zachariadis made an urgent referral to Dr. Dief and wrote that he strongly advised Mr. Georgoudakis to stay off work until further notice.

Therefore, Mr. Georgoudakis's testimony and the contemporaneous medical evidence confirms that he was physically unable to return to work as a subway operator up until mid-November, 2011, and, as of November 3, 2011, he was also psychologically unable to return to subway driving duties.

Whether or not he was offered modified work during the qualifying period is irrelevant, as the LOU provides only that the employee must show that during the qualifying period he is totally disabled from his own occupation. As outlined previously, there is no basis upon which to terminate his LTD benefits once they kick in as of December 30, 2011, based on either: (a) a lack of satisfactory continuing medical treatment and supervision or (b) any offer of suitable modified work.

In the alternative, Union counsel asked that should I determine that the Grievor was not under satisfactory medical treatment and supervision throughout the entire "own occupation" period, there is at the very least no question that he was under satisfactory medical treatment and supervision from December, 2011, to March, 2012, given that he was seeking medical treatment every few weeks: He saw Dr. Zachariadis on December 7, 2011, Dr. Dief on December 30, 2011, Dr. Zachariadis on February 10, 2012, and Dr. Dief on March 6, 2012.

In addition, counsel pointed out that during that period, Dr. Zachariadis had provided the unequivocal opinion that Mr. Georgoudakis was unable to return to work until further notice (in his December 7, 2011 note).

In terms of remedy, should the grievance be allowed, counsel requested:

1. A declaration that the TTC has breached the Collective Agreement and that the Grievor is entitled to LTD benefits in accordance with the Collective Agreement.
2. An award of LTD benefits from December 30, 2011 to December 30, 2013, including interest.
3. Special damages of 75% of his wages during the qualifying period.

The basis for this remedy is that, by satisfying the definition of “total disability” during the qualifying period (i.e. that he was totally disabled from his own occupation), Mr. Georgoudakis should be entitled to the equivalent value of the sick benefits he would have been entitled to during this period.

4. General damages in the amount of \$25,000 for lost peace of mind and mental distress.
5. \$5,000 in damages for the breach of his human rights.

THE EMPLOYER'S SUBMISSIONS

It is the position of the employer that the grievance should be dismissed because the Union has not discharged its onus of proving that the Grievor was totally disabled during the relevant period. The basis for the Grievor's claim of total disability is psychological. Most, if not all, of the medical evidence adduced in support of his claim is based on his self-reported symptoms and claimed restrictions.

In summary, the TTC takes the position that:

- (1) Medical evidence based on self-report is insufficient to establish a claim of total disability;

(2) Mr. Georgoudakis's fundamental lack of credibility renders any medical that is based on his self-reported symptoms and restrictions inherently unreliable to the point where it cannot substantiate total disability;

(3) In any event, Manulife's denial of LTD benefits based on the evidence provided by the Grievor, without taking into account his lack of credibility, is correct;

(4) In the further alternative, if the Grievor's claim is found to be substantiated, the \$50,000 settlement payment he received for lost income as the result of his car accident in 2012 must be offset against any LTD benefits for the period between February 12, 2012, (date of the car accident) and December 31, 2013.

In addressing the first point set out above, employer counsel argued that LTD entitlement cases are highly fact-specific and largely influenced by the medical evidence of disability. Counsel suggested that the following principles are supported by the case law:

- The onus of proof is on the Grievor/Union to establish that the Grievor is totally disabled.
- Objective medical evidence is required to discharge that onus.
- A medical opinion that is only based on subjective self-reports of the Grievor is to be given limited, if any, weight.

In *Lakehead District School Board and CUPE* (Luborsky, 2003), the Grievor claimed to be totally disabled as a result of "chronic whiplash syndrome" from any occupation. Arbitrator Luborsky held that a survey of the cases indicates that the determination of whether an employee is incapable of substantial employment is fact-specific and largely influenced by the medical evidence of disability as opposed to the subjective opinions of the Grievor [par. 118].

Employer counsel next referred to *City of Toronto v. CUPE Local 79* (Tims, 2012). In this case, the Union alleged that the Grievor was totally disabled on the basis of physical and psychological issues, including a diagnosis of PTSD by one of the Grievor's treating physicians as the result of her denial of LTD benefits in 2003 and childhood abuse [p. 255]. Arbitrator Tims found that while she accepted the Union's submission that the opinion of the treating psychiatrist who saw the Grievor monthly over many years warrants careful attention (even though this doctor did not testify in the hearing), the mere fact that the doctor had such long-standing professional relationship with her patient did not excuse the Arbitrator from assessing her evidence in light of the evidence in its entirety [par. 421].

Counsel pointed out that Arbitrator Tims rejected the Union's argument that the insurance company's psychiatric consultant cannot be regarded as independent. She stated that while she must be alert to the fact that he was retained and paid by the insurance company, in the end she concluded he was fair and measured in his assessment of the Grievor [par. 346]. Counsel suggested that Dr. Blank was fair and measured and that her underlying rationale was also clearly articulated in providing her opinion after the review of the medical file. Her opinion should carry significant weight.

In contrast, as was pointed out in this case, the Grievor's physicians (as in our case with Dr. Zachariadis, Dr Dief and Dr. Dermer) "may have strayed into the realm of advocacy on behalf of her patient" [p. 349] Further, the Arbitrator accepted the employer's argument that it is important in assessing the medical evidence to distinguish between mere recitation of what the Grievor told her doctor and a professional opinion [par. 349].

This is directly applicable to our case, because the ongoing deficiency in the medical reports is a lack of any demonstrated objectivity. All the medical notes and reports appear to rely exclusively on the Grievor's subjective self-report of his symptoms at the relevant time.

In the *Joseph Brant Memorial Hospital and ONA Case* (Bendel, 2014), the Grievor challenged the denial of her claim for LTD benefits. In this case, there was no objective medical evidence for illness. This was because, unlike PTSD, there is no objective medical test that exists to establish the cause or existence of fibromyalgia (chronic pain) [para. 209-211].

In stating that an applicant could receive LTD benefits in the absence of objective medical in that limited circumstance, Arbitrator Bendel stated in paragraph 212:

In the absence of objective medical evidence of disability, the credibility of the claimant is very much in issue. It could scarcely be otherwise: a claimant in such a case cannot expect that an employer or insurance company from which a substantial sum of money is being claimed will simply take at face value his or her assertion of being disabled.

Counsel next referred to *St. John's Rehabilitation Hospital and ONA* (McNamee, 2016) where Arbitrator McNamee reviewed the authorities of both parties and set out the following principles at paragraph 82:

a) The onus of proof of total disability is upon the claimant (or in this case upon ONA). Of course, the civil standard, i.e. balance of probabilities, will apply. *Plouffe v. Mutual Life Assurance Co. of Canada*; *Mathers v. Sun Life Assurance Co. of Canada*; and *Chaplin v. Sun Life Assurance Co. of Canada*;

...

c) The testimony of a doctor or other medical practitioner ought to be discounted if he or she has become an advocate for the insured and this provides less than objective testimony. Similarly, a medical report which is based solely on the subjective narrative of the insured should be given limited, if any, weight. *Chaplin v. Sun Life Assurance Co. of Canada*;

d) The testimony of a medical practitioner retained by an insurer should also be considered carefully to ensure that it is objective and free from bias. *Re Toronto (City)* and *C.U.P.E., Local 79 (Tims)*;

e) The question is not whether the insured suffers from pain or any particular affliction, but rather whether he or she is disabled from performing work which is required of them pursuant to the "own occupation" or, as applicable, the "any occupation" test..... *Ditomaso v. The Manufacturers Life Insurance Company*; *Brennand v. Sun Life Assurance Company of Canada*;

...

j) The evidence must be considered in its entirety, having regard to all relevant factors, including expertise, objectivity, fairness, thoroughness and reason. Re Toronto (City) and C.U.P.E., Local 79 (Tims);

k) In a proceeding of this nature it is not merely my function to conduct a hearing as to the correctness of the insurer's decision. Rather I am to conduct a review, in the nature of a hearing de novo as to the merits of the assertion that the claimant is or is not totally disabled. Re Joseph Brant Memorial Hospital and Ontario Nurses' Association; and

l) In appropriate circumstances, the failure or refusal of a claimant to participate in a return to work process may be relevant. Re Toronto (City) and C.U.P.E., Local 79 (Tims).

Counsel for the employer made lengthy submissions with regard to the credibility of the Grievor and provided numerous examples of the Grievor's lack of honesty or believability while testifying in these proceedings. She suggested that I should conclude that the Grievor lacks any credibility whatsoever. She pointed out that he lied in this very arbitration proceeding for no reason, when he easily could have told the truth. He has lied in other legal proceedings under oath for financial gain and to suit his purpose. Counsel argued that the Grievor has demonstrated a complete and blatant disregard for the truth in too many situations, whether it is for personal gain or not.

Having come to the conclusions I have earlier in this award, it is not necessary to set out the lengthy list of examples reviewed by counsel to substantiate her submission that the Grievor totally lacked any credibility.

Counsel for the employer next responded to the Union's arguments. She started by pointing out that in accordance with the LTD LOU, in order to even be eligible for LTD, you need to meet the qualifying period of 26 weeks of sick benefits before that. The medical notes from Dr. Zachariadis prove that the Grievor was not totally disabled in the

summer of 2011 and could return to modified duties. In addition, in the dismissal decision, it has already been found at p.70, 2nd paragraph, first sentence:

“In this case, the medical evidence makes it clear that the Grievor could have and should have returned to work on modified duties.”

The fact that the Grievor was not totally disabled and could have performed modified duties in the 26 weeks prior to December, 2011, disentitles the Grievor to sick benefits during this time period. Accordingly, the Grievor did not meet the qualifying period for LTD prior to December, 2011, as he did not meet the requirement for 26 weeks of sick benefits. He could have and should have returned to work, therefore that is sufficient to end this case.

In addition, counsel suggested that Section 1 of the LTD LOU confirms that the Grievor was supposed to establish that he “was under continuing medical supervision and treatment considered satisfactory to the TTC,”. He did not do so in her opinion. The fact that Mr. Georgoudakis was “referred” to specialists or programs does not constitute treatment in any way. In fact, it only highlights that there were programs available to him that his treating physicians wanted him to get and for whatever reason he did not take advantage of them. In terms of wait lists for programs, the point was that had he sought to participate in treatment options that he had been referred to from the beginning, the longer wait lists would not have been a problem. You can’t not put yourself on a wait list and then years later put wait lists forward as a reason for not being able to undergo treatment.

It was not the position of the employer that Mr. Georgoudakis should have seen Dr. Zachariadis more often but that he should have undergone the treatment that was being recommended. For what ever reason, he did not. For all the reasons set out above, the employer argued that the Grievor did not discharge the onus of proving that he was under “continuing medical supervision and **treatment considered satisfactory** to the TTC.”

The Union argued that there was medical evidence of psychological issues prior to December 7, 2011. Employer counsel pointed out that in the dismissal decision it had already been found that:

Mr. Georgoudakis went off work on June 29, 2011, because of a back injury. His WSIB claim was approved up to July 19, 2011. The Grievor's treating physician, Dr. Zachariadis, filled out a form confirming the back injury. There was no mention of any psychological issues at this point. Between the time that he went off work in June, 2011, and December, 2011, Mr. Georgoudakis spoke frequently with Ms. Stojicic and supplied medical reports from Dr. Zachariadis. At no time in his conversations with Ms. Stojicic did he indicate he was suffering from psychological issues nor did the reports he submitted indicate this.

Counsel pointed out that the Union did not argue that Dr. Blank was not objective in her assessment. In *St. John's Rehabilitation Hospital and ONA* (McNamee, 2016), Arbitrator McNamee observed: "The testimony of a medical practitioner retained by an insurer should also be considered carefully to ensure that it is objective and free from bias." Counsel suggested that Dr. Blank passed this test with flying colours.

UNION REPLY SUBMISSIONS

Counsel for the Union addressed the position taken by the employer that in accordance with the LTD LOU, in order to even be eligible for LTD, you need to meet the qualifying period of 26 weeks of sick benefits before that and that the Grievor was not totally disabled in the summer of 2011 and could have returned to modified duties. The TTC is saying that the Grievor failed to perform suitable modified work in the qualifying period but the Union disputes this. In addition, counsel suggested that the test was not whether he could have returned to modified duties but whether or not the Grievor was totally disabled in his own occupation during the qualifying period. She pointed out that the

evidence was clear that he could not perform his regular work driving a subway train during this time.

FINAL CONCLUSIONS

The parties were in dispute with regard to whether or not Mr. Georgoudakis met the definition of total disability and totally disabled as it is defined in Section 6 of the LTD LOU. The LOU provides:

...for the Qualifying Period and for the first 24 months immediately following the Qualifying Period, the employee is wholly and continuously disabled by illness or accidental bodily injury which prevents him or her performing any and every duty of the employee's normal occupation.

The Union argued that Mr. Georgoudakis was totally disabled from his own position in the qualifying period or in other words that it had been proved that he was totally disabled in his own occupation. She suggested that the medical evidence, taken as a whole, supported that on the balance of probabilities the Grievor was wholly and continuously disabled from his own occupation during the relevant period. Counsel asserted that none of the Grievor's treating physicians between 2011 and 2013 were of the medical opinion that he could perform the essential duties of a subway operator and/or could perform that work to the standard of a reasonable employee. She argued that the evidence was clear that he could not perform subway duties.

Counsel for the employer pointed out that in accordance with the LTD LOU, in order to even be eligible for LTD, you need to meet the qualifying period of 26 weeks of sick benefits before that. The LOU, under the heading Eligibility Requirements, notes that to be eligible to receive benefits a person must complete the qualifying period and the qualifying period is defined as 26 weeks of sick benefits, plus the exhaustion of Unemployment Insurance Commission (U.T.C.) benefits (if any), plus the exhaustion of all vacation pay entitlement (if any). The medical documentation provided to the TTC by Dr. Zachariadis proves that the Grievor was not totally disabled in the summer of 2011 and could have returned to modified duties. The fact that the Grievor was not totally disabled and could have performed modified duties in the 26 weeks prior to December, 2011, disentitles the Grievor to sick benefits during this time period. Accordingly, the

Grievor did not meet the qualifying period for LTD prior to December, 2011, as he did not meet the requirement for 26 weeks of sick benefits.

I am satisfied that the Grievor could have returned to modified duties at some point in the 26-week qualifying period. His own doctor confirmed that he could perform modified duties. It is clear is that he was not following the recommendations of his treating physician. It was not until December, 2011, that it was suggested that he was suffering from mental-health issues. Up to that point, his claims had been physical in nature and accommodated work was available and offered. The Grievor refused work during the qualifying period that in my view was suitable.

Counsel for the Union suggested that by November 3, 2011, his psychological symptoms were severe enough that he went to see Dr. Zachariadis, who provided a notation of PTSD. I have carefully reviewed the clinical note referred to. The note is very brief and in it Dr. Zachariadis states that Mr. Georgoudakis is still haunted by four incidents of suicide. He then writes 'Post Traumatic Stress Disorder". It is not clear to me whether Dr. Zachariadis is simply noting that Mr. Georgoudakis has suffered from PTSD in the past or whether he is coming to some conclusion based on what Mr. Georgoudakis is currently telling him. Given that nothing was said to the TTC about this, the vagueness and the brevity of the notation, I do not find this particular evidence to be helpful or conclusive.

Therefore, I do not conclude that the Grievor was totally disabled as it is defined in Section 6 of the LTD LOU for the qualifying period. He did not meet this requirement. However, I would also like to make it clear that even if I had concluded that he had met this requirement, I do not accept that for the first 24 months immediately following the

qualifying period Mr. Georgoudakis was wholly and continuously disabled by illness that prevented him from performing the essential duties, or as it is stated in the LOU, “any and every duty,” of his position as a subway operator.

Both parties made lengthy submissions with regard to the conclusions I should reach regarding the credibility of the Grievor. As I noted earlier in this decision I did not find Mr. Georgoudakis to be a credible witness. I found him to be evasive, vague, dishonest, argumentative and unresponsive when being cross-examined. Although I set out two incidents in particular, they were just two of many, many examples.

While counsel for the Union did an admirable job in trying to explain away or downplay aspects of the Grievor’s testimony that I have found to be particularly damning, I think that the testimony of the Grievor is beyond salvation. Accordingly, while I have carefully considered these submissions again I am not going to set them out as to do so would not be helpful or cause me to change my conclusions with regard to the testimony of the Grievor and his lack of credibility as noted earlier in this decision.

Had I come to a different conclusion with regard to the testimony of the Grievor, I might well have found that he was entitled to LTD for the period claimed. Although Mr. Georgoudakis was telling his doctors how severe his symptoms were, I simply do not believe him. His testimony was extremely self-serving. I do not believe that he was totally disabled. I believe that he exaggerated his symptoms and was not being truthful about his medical condition so that he could receive compensation from his employer pursuant to the LTD entitlements found in the Collective Agreement.

Given the conclusion reached above, it is not necessary to review and address the submissions of the parties with regard to whether or not Mr. Georgoudakis was under continuing medical supervision and treatment.

Accordingly, for all of the above-noted reasons, the grievance is dismissed.

I would like to thank both counsel for the work they did in this case. The Grievor was very difficult and argumentative at times. Counsel for the union did everything possible to put the best case forward on behalf of the Grievor. Both counsel handled the contentious situations as they arose with patience and professionalism. Both counsel did an excellent job representing their clients. The final submissions were thorough, thoughtful and helpful.

In the event there are any issues with regard to the interpretation of this award, I shall remain seized.

Dated at Toronto this 7th day of January, 2019

"Janice Johnston"
Janice Johnston
Arbitrator